



Name of Patient/Veteran

Patient/Veteran's Social Security Number

Date of examination:

IMPORTANT - THE DEPARTMENT OF VETERANS AFFAIRS (VA) **WILL NOT PAY OR REIMBURSE** ANY EXPENSES OR COST INCURRED IN THE PROCESS OF COMPLETING AND/OR SUBMITTING THIS FORM.

Note - The Veteran is applying to the U.S. Department of Veterans Affairs (VA) for disability benefits. VA will consider the information you provide on this questionnaire as part of their evaluation in processing the Veteran's claim. VA may obtain additional medical information, including an examination, if necessary, to complete VA's review of the Veteran's application. VA reserves the right to confirm the authenticity of ALL completed questionnaires. **It is intended that this questionnaire will be completed by the Veteran's healthcare provider.**

Are you completing this Disability Benefits Questionnaire at the request of:

☐ Veteran/Claimant☐ Third party (please list name(s) of organization(s) or individual(s))☐ Other: please describe

Are you a VA Healthcare provider?

☐ Yes☐ No

Is the Veteran regularly seen as a patient in your clinic?

☐ Yes☐ No

Was the Veteran examined in person?

☐ Yes☐ No

If no, how was the examination conducted?

EVIDENCE REVIEW

Evidence reviewed:

☐ No records were reviewed☐ Records reviewed

Please identify the evidence reviewed (e.g. service treatment records, VA treatment records, private treatment records) and the date range.

SECTION 1 - DIAGNOSIS

Note: These are condition(s) for which an evaluation has been requested on the exam request form (Internal VA) or for which the Veteran has requested medical evidence be provided for submission to VA.

1A. List the claimed condition(s) that pertain to this questionnaire:

Note: These are the diagnoses determined during this current evaluation of the claimed condition(s) listed above. If there is no diagnosis, if the diagnosis is different from a previous diagnosis for this condition, or if there is a diagnosis of a complication due to the claimed condition(s), explain your findings and reasons in the Remarks section. Date of diagnosis can be the date of the evaluation if the clinician is making the initial diagnosis or an approximate date determined through record review or reported history.

1B. Select diagnoses associated with the claimed condition(s) (check all that apply):

☐ The Veteran does not have a current diagnosis associated with any claimed condition(s) listed above. (Explain your findings and reasons in the Remarks section)

☐ Parkinson's disease	ICD code: _____	Date of diagnosis: _____
☐ Parkinsonism (identify type if known)	ICD Code: _____	Date of diagnosis: _____
☐ Progressive supranuclear palsy (PSP)	ICD code: _____	Date of diagnosis: _____
☐ Multiple system atrophy (MSA) (Shy-Drager syndrome)	ICD Code: _____	Date of diagnosis: _____
☐ Corticobasal degeneration (CBD)	ICD Code: _____	Date of diagnosis: _____
☐ Vascular Parkinsonism	ICD Code: _____	Date of diagnosis: _____
☐ Dementia with Lewy bodies (Also complete appropriate Mental Disorders questionnaire)	ICD Code: _____	Date of diagnosis: _____
☐ Drug-induced Parkinsonism	ICD Code: _____	Date of diagnosis: _____
☐ Other Specify: _____	ICD Code: _____	Date of diagnosis: _____

SECTION 2 - MEDICAL HISTORY

2A. Describe the history (including onset and course) of the Veteran's Parkinson's disease or Parkinsonism (brief summary):

SECTION 3 - DOMINANT HAND

3A. Dominant hand: ☐ Right ☐ Left ☐ Ambidextrous

SECTION 4 - MOTOR MANIFESTATIONS

4A. Hypomimia (facial masking):

Is there reported/documented hypomimia due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", indicate side affected and severity

☐ Right	☐ Mild	☐ Moderate	☐ Severe
☐ Left	☐ Mild	☐ Moderate	☐ Severe

4B. Stooped posture:

Is there reported/documented stooped posture due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", indicate severity

☐ Mild ☐ Moderate ☐ Severe

4C. Balance impairment:

Is there reported/documented balance impairment due to Parkinson's Disease/Parkinsonism or its treatment?

- ☐ Yes ☐ No If "Yes", indicate severity
- ☐ Mild ☐ Moderate ☐ Severe

4D. Bradykinesia or slowed motion (difficulty initiating movement, "freezing", short shuffling steps):

Is there reported/documented bradykinesia or slowed motion due to Parkinson's Disease/Parkinsonism or its treatment?

- ☐ Yes ☐ No If "Yes", identify extremities affected
- ☐ Right upper extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe
- ☐ Left upper extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe
- ☐ Right lower extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe
- ☐ Left lower extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe

4E. Tremors (characteristic hand shaking, "pill rolling"):

Is there reported/documented tremors due to Parkinson's Disease/Parkinsonism or its treatment?

- ☐ Yes ☐ No If "Yes", identify extremities affected
- ☐ Right upper extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe
- ☐ Left upper extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe
- ☐ Right lower extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe
- ☐ Left lower extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe

4F. Muscle rigidity and stiffness:

Is there reported/documented muscle rigidity and/or stiffness due to Parkinson's Disease/Parkinsonism or its treatment?

- ☐ Yes ☐ No If "Yes", identify extremities affected
- ☐ Right upper extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe
- ☐ Left upper extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe
- ☐ Right lower extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe
- ☐ Left lower extremity (If checked, indicate severity) ☐ Mild ☐ Moderate ☐ Severe

4G. Muscle weakness:

Does the Veteran have any muscle weakness in the upper and/or lower extremities due to Parkinson's Disease/Parkinsonism or its treatment?

- ☐ Yes ☐ No If "Yes", rate strength according to the following scale for the extremity(ies) involved

0/5 No muscle movement	1/5 Visible muscle movement, but no joint movement	2/5 No movement against gravity
3/5 No movement against resistance	4/5 Less than normal strength	5/5 Normal strength

Shoulder flexion:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Shoulder abduction:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Elbow Flexion:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Elbow Extension:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Wrist Flexion:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5

Wrist Extension:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Grip:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Pinch: (thumb to index finger)	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Hip flexion:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Hip abduction:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Knee Flexion:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Knee Extension:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Ankle Plantar Flexion:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
Ankle Dorsiflexion:	Right:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5
	Left:	☐ 5/5	☐ 4/5	☐ 3/5	☐ 2/5	☐ 1/5	☐ 0/5

4H. Reflexes:

Does the Veteran have any impaired reflexes in the upper and/or lower extremities due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", rate reflexes according to the following scale

0 Absent 1+ Decreased 2+ Normal 3+ Increased without clonus 4+ Increased with clonus

Biceps:	Right:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+
	Left:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+
Triceps:	Right:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+
	Left:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+
Brachioradialis:	Right:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+
	Left:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+
Knee:	Right:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+
	Left:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+
Ankle:	Right:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+
	Left:	☐ 0	☐ 1+	☐ 2+	☐ 3+	☐ 4+

4I. Gait:

Does the Veteran have an abnormal gait due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If yes, describe: _____

4J. Muscle atrophy:

Does the Veteran have muscle atrophy due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", indicate location

☐ Right upper extremity

☐ Left upper extremity

☐ Right lower extremity

☐ Left lower extremity

When possible, for each instance of muscle atrophy, provide measurements in centimeters of normal side and atrophied side, measured at maximum muscle bulk: If more than one location is atrophied document using the same format.

Location: _____ Normal side: _____ cm Atrophied side: _____ cm

SECTION 5 - DIGESTIVE MANIFESTATIONS

Note 1: Findings must be documented by barium swallow, computerized tomography (CT), or esophagogastroduodenoscopy (EGD). (Indicate date of study in Section XIII - Remarks)

Note 2: Recurrent esophageal stricture is defined as the inability to maintain target esophageal diameter beyond 4 weeks after the target diameter has been achieved. Refractory esophageal stricture is defined as the inability to achieve target esophageal diameter despite receiving no fewer than 5 dilatation sessions performed at 2-week intervals.

Note 3: "Undernutrition" means a deficiency resulting from insufficient intake of one or multiple essential nutrients, or the inability of the body to absorb, utilize, or retain such nutrients. Undernutrition is characterized by failure of the body to maintain normal organ functions and healthy tissues. Signs and symptoms may include: loss of subcutaneous tissue, edema, peripheral neuropathy, muscle wasting, weakness, abdominal distention, ascites, and Body Mass Index below normal range.

Note 4: "Substantial weight loss" means involuntary loss greater than 20 percent of an individual's baseline with sustained for three months with diminished quality of self-care or work tasks. "Baseline weight" means the clinically documented average weight for the two-year period preceding the onset of illness or, if relevant, the weight recorded at the Veteran's most recent discharge physical. If neither of these weights is available or currently relevant, then use ideal body weight as determined by either the Hamwi formula or Body Mass Index tables, whichever is most favorable to the Veteran.

5A. Chewing dysfunction:

Is there reported/documented chewing dysfunction due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", indicate severity

☐ Mild ☐ Moderate ☐ Severe

5B. Swallowing dysfunction:

Is there reported/documented swallowing dysfunction due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", check all that apply

☐ Dysphagia (if checked, answer the following question)

Is daily medication required? ☐ Yes ☐ No

☐ Documented history of esophageal stricture(s) (see Note 1 above) (if checked, answer the following)

Has the esophageal stricture(s) been recurrent? (see Note 2 above) ☐ Yes ☐ No

Has the esophageal stricture(s) been refractory? (see Note 2 above) ☐ Yes ☐ No

Identify any applicable symptoms/treatment related to the history of esophageal stricture(s):

☐ Aspiration

☐ Undernutrition (see Note 3 above)

☐ Substantial weight loss (see Note 4 above)

☐ Treatment with surgical correction (Indicate date and type of surgery in Section XIII - Remarks)

☐ Treatment with a percutaneous esophago-gastrointestinal tube (PEG tube) (Indicate date of procedure in Section XIII - Remarks)

☐ Requiring esophageal stent placement

☐ Requiring dilatation (if checked indicate frequency and list most recent dates):

☐ No more than 2 times a year ☐ 3 or more times a year

Was there dilatation utilizing steroids at least 1 time per year?

☐ Yes ☐ No

Date of dilatation _____ Date of dilatation _____ Date of dilatation _____

☐ Other Specify: _____

5C. Gastrointestinal symptoms:

Is there reported/documented gastrointestinal symptoms due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", check all that apply

☐ Constipation

☐ Change in stool frequency

☐ Change in stool form

☐ Altered stool passage (straining and/or urgency)

☐ Mucorrhea

☐ Abdominal bloating

☐ Subjective distention

☐ Other Specify: _____

☐ Abdominal pain related to defecation (if checked, indicate frequency during the previous 3 months)

☐ None ☐ At least once ☐ At least 3 days per month ☐ At least 1 day per week

5D. Sphincter control:

Is there reported/documented impairment of sphincter control due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", check all that apply

☐ Loss of sphincter control (If checked, indicate severity)

Note: Complete or partial loss of sphincter control refers to the inability to retain or expel stool at an appropriate time and place.

- ☐ Complete loss of sphincter control
- ☐ Partial loss of sphincter control
- ☐ History of loss of sphincter control, currently asymptomatic

☐ Bowel incontinence to solids and/or liquids (If checked, indicate frequency)

- ☐ Less than once every six months, which requires wearing a pad at least once every six months
- ☐ At least once every six months, which requires wearing a pad at least once every six months
- ☐ Two or more times per month, which requires wearing a pad two or more times per month
- ☐ Two or more times per week, which requires wearing a pad two or more times per week
- ☐ Two or more times per day, which requires changing a pad two or more times per day

☐ Physician-prescribed bowel program (If checked, indicate responsiveness)

- ☐ Fully responsive
- ☐ Partially responsive
- ☐ Not responsive

Indicate the bowel program requirements (Check all that apply)

- ☐ Special diet
- ☐ Medication (If checked, are the prescribed medication(s) beyond laxative use?)
- ☐ Yes ☐ No
- ☐ Digital stimulation
- ☐ Surgery (If checked, provide the date of surgery or anticipated date of surgery)

Date: _____

☐ Other Describe: _____

SECTION 6 - URINARY MANIFESTATIONS

6A. Urinary leakage:

Is there reported/documentated urinary leakage due to Parkinson's Disease/Parkinsonism or its treatment?

- ☐ Yes ☐ No If "Yes", indicate severity
- ☐ Requires wearing of absorbent materials (if checked, indicate how often they must be changed)
- ☐ Less than 2 times a day ☐ 2 to 4 times a day ☐ More than 4 times a day
- ☐ Requires the use of an appliance If checked, describe: _____

6B. Urinary frequency:

Is there reported/documentated increased urinary frequency due to Parkinson's Disease/Parkinsonism or its treatment?

- ☐ Yes ☐ No If "Yes", indicate frequency
- ☐ Daytime voiding interval greater than 3 hours ☐ Nighttime awakening to void less than 2 times
- ☐ Daytime voiding interval between 2 and 3 hours ☐ Nighttime awakening to void 2 times
- ☐ Daytime voiding interval between 1 and 2 hours ☐ Nighttime awakening to void 3 to 4 times
- ☐ Daytime voiding interval less than 1 hour ☐ Nighttime awakening to void 5 or more times

6C. Obstructive voiding:

Is there reported/documentated obstructive voiding due to Parkinson's Disease/Parkinsonism or its treatment?

- ☐ Yes ☐ No If "Yes", check all that apply
- ☐ Hesitancy (If checked, is hesitancy marked?)
- ☐ Yes ☐ No
- ☐ Slow or weak stream (If checked, is stream markedly slow or weak?)
- ☐ Yes ☐ No
- ☐ Decreased force of stream (If checked, is force of stream markedly decreased?)
- ☐ Yes ☐ No
- ☐ Stricture disease (If checked, indicate frequency of dilatation)
- ☐ Dilatation 1 to 2 times per year ☐ Periodic dilatation every 2 to 3 months ☐ Not requiring dilatation
- ☐ Recurrent urinary tract infections secondary to obstruction
- ☐ Uroflowmetry peak flow rate less than 10 cc/sec
- ☐ Post void residuals greater than 150cc
- ☐ Urinary retention requiring intermittent or continuous catheterization

6D. Recurrent urinary tract infections:

Is there reported/documentated recurrent urinary tract infections due to Parkinson's Disease/Parkinsonism or its treatment?

- ☐ Yes ☐ No If "Yes", check all that apply
- ☐ No treatment
- ☐ Suppressive drug therapy (if checked, indicate frequency)
- ☐ Lasting 6 months or longer ☐ For less than 6 months
- ☐ Hospitalization (if checked, indicate frequency of hospitalizations per year)
- ☐ 1 or 2 per year ☐ More than 2 per year
- ☐ Drainage by stent or nephrostomy tube
- ☐ Continuous intensive management required
- ☐ Other management/treatment not listed above

For all options checked above, list medications/management/treatments used for urinary tract infection and indicate dates for courses of treatment over the past 12 months:

SECTION 7 - OTHER MANIFESTATIONS

7A. Speech changes (monotone, slurring words, soft or rapid speech):

Is there reported/documented speech changes or is the Veteran's speech otherwise abnormal due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", check all that apply

- ☐ Hoarseness
- ☐ Inflammation of vocal cords or mucous membranes
- ☐ Thickening of vocal cords
- ☐ Nodules of vocal cords
- ☐ Polyps of vocal cords
- ☐ Submucous infiltration of vocal cords
- ☐ Constant inability to speak above a whisper
- ☐ Constant inability to communicate by speech
- ☐ Otherwise abnormal Describe: _____

7B. Sense of smell:

Is there reported/documented loss of sense of smell due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", indicate severity
☐ Partial ☐ Complete

7C. Sense of taste:

Is there reported/documented loss of sense of taste due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", indicate severity
☐ Partial ☐ Complete

7D. Sleep disturbance:

Is there reported/documented sleep disturbance due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No If "Yes", check all that apply

- ☐ Insomnia
- ☐ Signs and/or symptoms of sleep apnea or a sleep apnea-like condition (if selected, check all that apply)

Note: If signs and/or symptoms of sleep apnea or a sleep apnea-like condition are due to Parkinson's Disease/Parkinsonism or its treatment, a sleep study is not indicated.

- ☐ Persistent daytime hypersomnolence
- ☐ Requires use of breathing assistance device
- ☐ Chronic respiratory failure with carbon dioxide retention or cor pulmonale
- ☐ Requires tracheostomy
- ☐ Other Specify: _____

Is there reported/documented erectile dysfunction or female sexual arousal disorder (FSAD) due to Parkinson's Disease/Parkinsonism or its treatment?

☐ Yes ☐ No

8A. Does the Veteran have signs or symptoms of depression, cognitive impairment or dementia, or any other mental health condition(s) attributable to Parkinson's disease/Parkinsonism and/or its treatment?

☐ Yes ☐ No If yes, schedule with appropriate provider to complete a Mental Disorders Questionnaire

Note: The intention of this section is to permit the examiner to quantify the level of remaining function; it is not intended to inquire whether the Veteran should undergo an amputation with fitting of a prosthesis. For example, if the functions of grasping (hand) or propulsion (foot) are as limited as if the Veteran had an amputation and prosthesis, the examiner should check "yes" and describe the diminished functioning. The question simply asks whether the functional loss is to the same degree as if there were an amputation of the affected limb.

☐ Yes, functioning is so diminished that amputation with prosthesis would equally serve the Veteran.

☐ No

If "Yes", indicate extremities for which this applies: ☐ Right upper ☐ Left upper ☐ Right lower ☐ Left lower

[illegible]

10A. Does the Veteran use any assistive devices?

☐ Yes ☐ No

If yes, identify the assistive devices used. Check all that apply and indicate frequency.

☐ Wheelchair	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Brace(s)	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Crutch(es)	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Cane(s)	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Walker	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant
☐ Other:	Frequency of use:	☐ Occasional	☐ Regular	☐ Constant

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SECTION 11 - OTHER PERTINENT PHYSICAL FINDINGS, COMPLICATIONS, CONDITIONS, SIGNS, SYMPTOMS, AND SCAR

11A. Does the Veteran have any other pertinent physical findings, complications, conditions, signs, or symptoms, including any subjective symptoms not capable of objective verification (i.e., headaches, dizziness, fatigability), related to any conditions listed in the diagnosis section above?

☐ Yes ☐ No

If "Yes", describe (brief summary):

11B. Does the Veteran have any scars or other disfigurement (of the skin) related to any conditions or to the treatment of any conditions listed in the diagnosis section?

☐ Yes ☐ No If "Yes", also complete the appropriate dermatological questionnaire.

SECTION 12 - FUNCTIONAL IMPACT

Note: Provide the impact of only the diagnosed condition(s), without consideration of the impact of other medical conditions or factors, such as age.

12A. Regardless of the Veteran's current employment status, do the conditions listed in the diagnosis section impact his/her ability to perform any type of occupational task (such as standing, walking, lifting, sitting, etc.)?

☐ Yes ☐ No

If yes, describe the functional impact of each condition, providing one or more examples:

SECTION 13 - REMARKS	
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13A. Remarks (if any – please identify the section to which the remark pertains when appropriate).	
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SECTION 14 - EXAMINER'S CERTIFICATION AND SIGNATURE

CERTIFICATION - To the best of my knowledge, the information contained herein is accurate, complete and current.

PENALTY: The law provides severe penalties which include fine or imprisonment, or both, for the willful submission of any statement or evidence of a material fact, knowing it to be false, or for the fraudulent acceptance of any payment to which you are not entitled.

14A. Examiner's signature:

14B. Examiner's printed name and title (e.g. MD, DO, DDS, DMD, Ph.D, Psy.D, NP, PA-C):

14C. Examiner's area of practice/specialty (e.g. Cardiology, Orthopedics, Psychology/Psychiatry, General Practice):

14D. Date signed:

14E. Examiner's phone/fax numbers:

14F. National Provider Identifier (NPI) number:

14G. Medical license number and state:	

14H. Examiner's address: